

**HOW TO USE THIS SET**

20 questions, mixed difficulty, 10 case-based. Attempt every question before checking the answer key on the last page.

● 4 Easy

● 10 Medium

● 6 Hard

1.**EASY**

Achalasia is caused by absence of which structure in the body of the esophagus?

- A. Myenteric plexus
- B. Submucosal venous plexus
- C. Lower esophageal sphincter
- D. Muscularis mucosae
- E. Squamous epithelium

2.**EASY**

The commonest benign esophageal neoplasm is:

- A. Adenoma
- B. Hemangioma
- C. Leiomyoma
- D. Lipoma
- E. Squamous papilloma

3.**EASY**

The commonest presenting symptom of esophageal carcinoma is:

- A. Dysphagia
- B. Odynophagia
- C. Aspiration
- D. Hemorrhage
- E. Weight loss

4.**EASY**

Esophageal varices are due to cirrhosis and portal hypertension in approximately what proportion of cases?

- A. 10%
- B. 25%
- C. 50%
- D. 75%
- E. >90%

5.**MEDIUM****CASE**

A 34-year-old man from South America has progressive dysphagia. Manometry shows increased LES tone, incomplete LES relaxation and aperistalsis.

Which cause of this disorder is named in the lecture?

- A. Portal hypertension
- B. Chagas disease (*Trypanosoma cruzi*)
- C. Herpes simplex virus
- D. *Candida albicans*
- E. Plummer-Vinson syndrome

6.

MEDIUM CASE

A cirrhotic patient undergoes endoscopy, which shows dilated submucosal veins in the distal esophagus.

Above roughly what diameter do these veins carry a risk of rupture?

- A. 2 mm
- B. 3 mm
- C. 5 mm
- D. 8 mm
- E. 12 mm

7.

MEDIUM CASE

After a bout of acute alcohol intoxication with severe retching, a man vomits blood. Endoscopy shows longitudinal mucosal tears near the GE junction.

What is the diagnosis?

- A. Mallory-Weiss tear
- B. Esophageal varices
- C. Eosinophilic esophagitis
- D. Boerhaave syndrome
- E. Candidal esophagitis

8.

MEDIUM CASE

After severe vomiting, a man develops severe chest pain, tachypnea and shock.

What complication accounts for his presentation?

- A. A tracheoesophageal fistula
- B. Barrett dysplasia
- C. Severe mediastinitis after transmural rupture
- D. Achalasia crisis
- E. Variceal hemorrhage

9.

MEDIUM CASE

An immunosuppressed patient has odynophagia. Endoscopy shows adherent gray-white plaques over the mucosa.

The plaques are composed of:

- A. Multinucleate squamous cells
- B. Glandular metaplastic epithelium
- C. Densely matted fungal hyphae and inflammatory cells
- D. Eosinophils in transverse ridges
- E. Necrotic tumour

10.

MEDIUM CASE

A patient receiving immunosuppression has odynophagia. Endoscopy shows shallow vesicles and ulcers that have coalesced into extensive areas.

Which biopsy finding would confirm the cause?

- A. Matted fungal hyphae forming pseudomembranes
- B. Numerous eosinophils far from the GEJ
- C. Glandular metaplasia
- D. Multinucleate squamous cells containing herpesvirus nuclear inclusions
- E. Basal zone hyperplasia with elongated papillae

11.

MEDIUM CASE

An atopic child with peripheral eosinophilia has food impaction. Endoscopy shows transverse ridges resembling the trachea.

Where would the biopsy show numerous eosinophils?

- A. Only in the muscularis propria
- B. Within the lamina propria papillae only
- C. In the superficial epithelium, at sites far from the GEJ
- D. Only in the basal zone near the GEJ
- E. They are absent in this disease

12.

MEDIUM CASE

A 52-year-old man with heartburn has a biopsy showing basal zone hyperplasia, elongation of lamina propria papillae, and eosinophils then neutrophils in the epithelium.

Which diagnosis fits best?

- A. Mallory-Weiss tear
- B. Herpes esophagitis
- C. Candidal esophagitis
- D. Reflux esophagitis
- E. Eosinophilic esophagitis

13.

MEDIUM

Which of the following is listed as a factor predisposing to GERD?

- A. Reduced gastric volume
- B. Hiatal hernia
- C. Rapid gastric emptying
- D. Weight loss
- E. Increased LES tone

14.

MEDIUM

Which intermediate stage lies between Barrett metaplasia and adenocarcinoma?

- A. Basal zone hyperplasia
- B. Mediastinitis
- C. Ulceration
- D. Stricture
- E. Dysplasia

15.

HARD CASE

A 62-year-old man from a Western country with long-standing GERD has a tumour in the distal esophagus invading the adjacent gastric cardia.

Which statement best fits this tumour?

- A. It is most likely an adenocarcinoma arising from Barrett esophagus, and its incidence is increasing
- B. It is a squamous cell carcinoma, the commonest type worldwide
- C. It is a leiomyoma
- D. It typically involves the middle third
- E. It arises from squamous epithelium after caustic injury

16.

HARD

Which is NOT a listed risk factor for esophageal SCC?

- A. Achalasia
- B. Barrett esophagus
- C. Plummer-Vinson syndrome
- D. Previous mediastinal radiation
- E. Caustic esophageal injury

17.

HARD CASE

A patient has an esophageal carcinoma in the lower third.

Which lymph nodes are the expected sites of metastasis?

- A. Tracheobronchial nodes
- B. Paratracheal nodes
- C. Mediastinal nodes
- D. Cervical nodes
- E. Gastric and celiac nodes

18.

HARD

A biopsy from the mid-esophagus of an atopic child shows numerous intraepithelial eosinophils far from the GEJ. Which feature most argues against reflux esophagitis?

- A. The number and distance of the eosinophils from the GEJ
- B. The presence of any eosinophils
- C. A history of atopy being irrelevant
- D. The presence of basal zone hyperplasia
- E. The child's young age

19.

HARD

Which is a listed cause of achalasia-like (pseudo-achalasia) disease?

- A. Barrett esophagus
- B. Candida infection
- C. Hiatal hernia
- D. Diabetic autonomic neuropathy
- E. Mallory-Weiss tear

20.

HARD

Which is NOT a listed complication of reflux esophagitis?

- A. Stricture
- B. Achalasia
- C. Esophageal ulceration
- D. Barrett esophagus
- E. Hematemesis and melena

ANSWER KEY & RATIONALE

Don't peek until you've committed to an answer on every question.

Q	Ans	Why
1	A	Absence of the myenteric plexus gives increased LES tone and functional obstruction.
2	C	Benign tumours are epithelial (papilloma) or mesenchymal; leiomyoma is the commonest.
3	A	Dysphagia is the commonest symptom; the disease usually presents at an advanced stage.
4	E	Liver cirrhosis and portal hypertension (>90%), mostly alcohol-induced.
5	B	Secondary achalasia: Chagas disease destroys the myenteric plexus.
6	C	Varices exceeding 5 mm may rupture, leading to life-threatening bleeding.
7	A	Longitudinal mucosal tears near the GE junction after retching = Mallory-Weiss; they present with hematemesis.
8	C	Boerhaave syndrome: transmural rupture of the distal esophagus producing severe mediastinitis; usually fatal.
9	C	Candidal esophagitis: adherent gray-white pseudomembranes of matted fungal hyphae and inflammatory cells.
10	D	Herpes esophagitis: shallow vesicles and ulcers that may coalesce; multinucleate squamous cells with herpesvirus nuclear inclusions.
11	C	Eosinophilic esophagitis: large numbers of eosinophils infiltrate the superficial epithelium and at sites far from the GEJ.
12	D	Basal zone hyperplasia, elongated papillae and eosinophils then neutrophils in the epithelium characterise reflux esophagitis.
13	B	GERD follows conditions that decrease LES tone and/or increase abdominal pressure: alcohol, tobacco, obesity, CNS antidepressants, pregnancy, hiatal hernia, delayed gastric emptying, increased gastric volume.
14	E	Dysplasia is the intermediate stage between metaplasia and malignancy in Barrett esophagus.
15	A	Most esophageal adenocarcinomas arise from Barrett esophagus, usually in the distal third and may invade the gastric cardia; incidence is rising worldwide.
16	B	Barrett esophagus is the precursor of adenocarcinoma, not SCC. The others are listed SCC risk factors.
17	E	Lower third: gastric and celiac LN. Upper third: cervical. Middle third: mediastinal, paratracheal, tracheobronchial.
18	A	Eosinophils occur in both diseases; numerous eosinophils far from the GEJ (with transverse ridges) point to eosinophilic esophagitis rather than reflux.
19	D	Achalasia-like disease: diabetic autonomic neuropathy; infiltration by malignancy, amyloidosis, sarcoidosis; lesions of dorsal motor nuclei (polio, surgical ablation).
20	B	Listed complications: ulceration, hematemesis and melena, stricture, Barrett esophagus. Achalasia is a motility disorder from absent myenteric plexus.

Pathology GIT Block · MEDucation by Mattin Majid · MCQs written to test LG2 content — not sourced from a past paper.

Recalled and source questions are treated as evidence of what gets asked, not as a source of correct answers — verify against your lecture notes.